term are quite different from the meaning psychologists give to the term personality.

Ways of Defining and Thinking About Personality

When psychologists define *personality*, they tend to refer to qualities within a person, characteristics of a person's behavior, or both. In a now-famous definition, psychologist Gordon Allport (1937) mentioned both inner qualities and behavior, but he emphasized the inner qualities: "Personality is the dynamic organization within the individual of those psychophysical systems that determine his unique adjustments to his environment" (p. 48). In a more recent definition, psychologist Walter Mischel (1976) mentioned both inner processes and behavior but emphasized behavior. Personality, he wrote, consists of "the distinctive patterns of behavior (including thoughts and emotions) that characterize each individual's adaptation to the situations of his or her life" (p. 2). No single definition of *personality* is acceptable to all psychologists. However, most agree that personality includes the behavior patterns a person shows across situations or the psychological characteristics of the person that lead to those behavior patterns.

Personality has been studied in a number of different ways. Some have developed broad theories to explain the origins and makeup of personality. Others have focused only on one or two issues, such as the influence of heredity on personality.

The first approach, theory construction, was popular for many years. As a result, we have many broad personality theories; much of this chapter will focus on them. Most of these broad theories can be grouped into four categories: (1) *type and trait approaches*, which focus on people's characteristics—stubbornness, shyness, and so forth—and how these characteristics are organized into systems; (2) *dynamic approaches*, which emphasize on-going interactions among motives, impulses, and psychological processes; (3) *learning and behavioral approaches*, which emphasize the ways habits are acquired through basic conditioning or learning processes; and (4) *humanistic approaches*, which emphasize the self and the importance of the individual's subjective view of the world.

Which of these theories is best? Which gives the most accurate picture of "true" personality? The honest answer is that we do not really know; one reason is that we have not yet found acceptable ways to test entire theories. We can test specific implications of particular theories, one or two at a time, but this is a slow process, and it has not yet brought us anywhere near a final answer. What does seem likely, though, is that there is some truth and some useful insight in many of the major personality theories. In this chapter we will delve into several of these theories, focusing first on type and trait approaches and then moving to the other three categories listed above.

Type and Trait Theories of Personality

Type and trait theories of personality both focus on people's personal characteristics. However, various type theorists and trait theorists differ in

the ways they use those characteristics to describe people. We will begin to examine these differences by returning briefly to George and Crystal and considering what "types" of people they are.

TYPE THEORIES

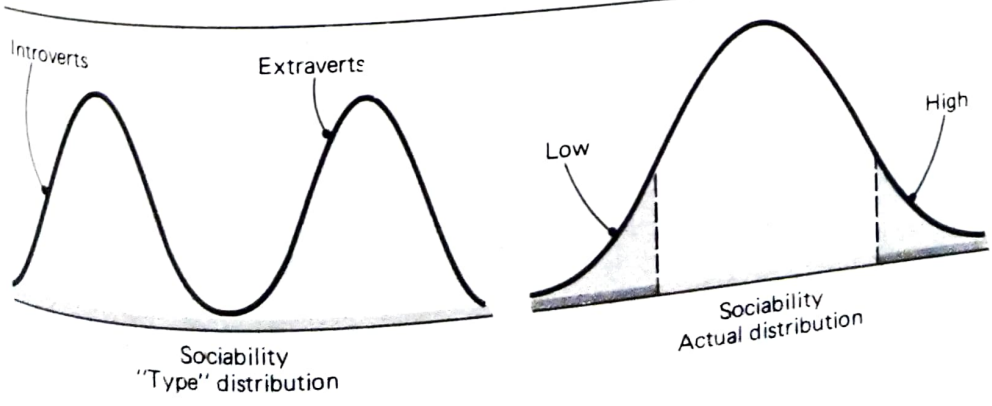
Reading about George and Crystal, you may have found yourself classifying them into "types." You may have seen Liddy as the "gung-ho type"; Crystal may have seemed like the "spontaneous type," or perhaps the "energetic type." Classifying people into types is one device many of us use to try to make sense out of others' behavior and to anticipate how they will act in the future. The notion that people can be classified into types is one of the oldest ideas about personality—over 2,000 years old, in fact.

One of the first type theories that we know of was proposed about 400 B.C. by Hippocrates, a Greek physician now known as the father of medicine. He grouped people into four temperament types: sanguine—cheerful, vigorous, confidently optimistic; melancholic—depressed, morose; choleric—hot-tempered; and phlegmatic—slow-moving, calm, unexcitable. Since the time of Hippocrates, countless other ways of grouping people into types have been tried. The groupings or sets of types are called typologies; they can be found in the folk wisdom of most cultures. In American culture, for example, people are often classified as leaders or followers, liberals or conservatives, losers or winners, and so forth; on some college campuses, the typologies include jocks, preps, Greeks, necks, heads, and punks. Typologies have been with us for centuries, and they are very much with us today.

What are these types that we put together to form typologies? A *type* is simply a class of individuals said to share a common collection of characteristics. For example, *introverts* could be described as people who share characteristics such as shyness, social withdrawal, and a tendency not to talk much; while *extraverts* share a tendency to be outgoing, friendly, and talkative. Usually, sharp typologies do not work well scientifically. A major reason is that almost every dimension of personality—friendliness, sociability, and so on—is distributed according to a normal bell-shaped curve, as discussed in Chapter 13, page 534. If people fell naturally into distinctly different types, we would see not bell-shaped curves but humps corresponding to the types, as shown in the left side of Figure 14.1. On most human characteristics and especially psychological characteristics—such as sociability—people tend to be bunched near the middle of the range, and only a few stand out at the extremes, as shown in the right side of Figure 14.1.

If people clustered into types, measures of their characteristics should also tend to cluster. The distriubtion on the left shows, in exaggerated form, how one kind of measure would be distributed if everyone could be categorized as either *introvert* or *extravert*. On the right, the bell-shaped, normal distribution shows that most people are reasonably close to average levels of sociability, with only the extremes of sociability seeming like "types" to us. Although we can all call to mind people who are extremely introverted or extremely extraverted, in reality most people are neither.

Figure 14.1

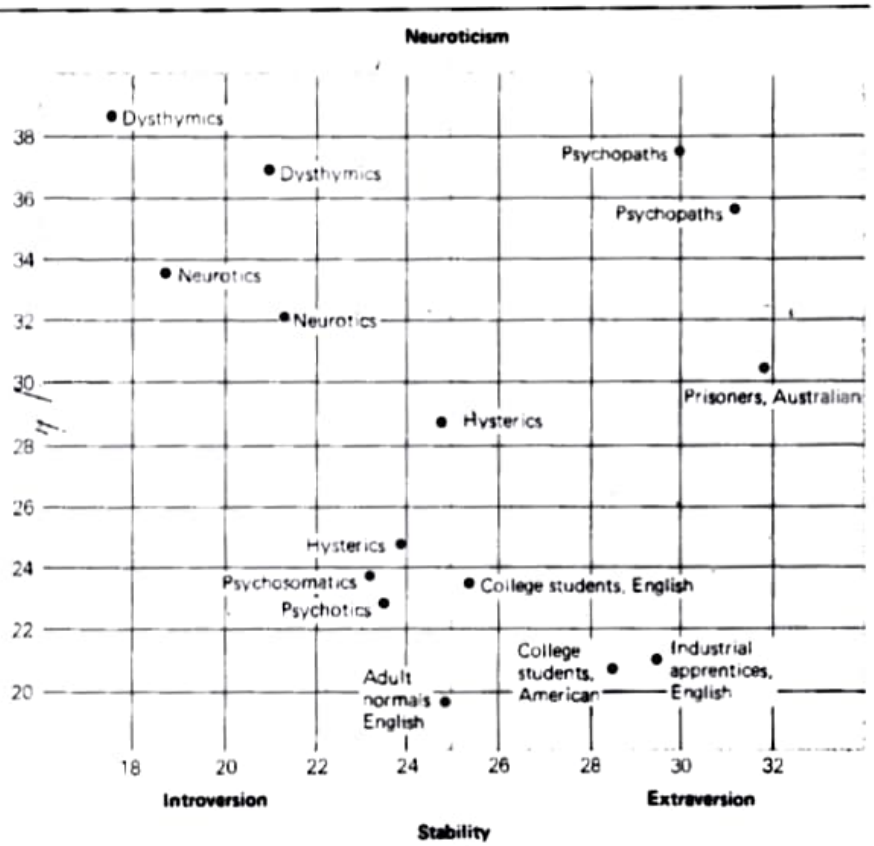


Since people do not fall naturally into distinct personality types, psychologists who want to develop type theories have to take special steps. One approach has been to develop theories about specific types by treating each type as a personality dimension. Individuals can then be scored or rated to determine their position on each dimension—that is, how close they come to fitting one of the types. One theorist who used this approach is H. J. Eysenck. We will examine his theory first.

Eysenck's hierarchical theory Eysenck (1967) identified the major components of personality as a small number of personality types. Each type is made up of a set of personality characteristics. For example, people who fit Eysenck's extraverted type are said to have such characteristics as sociability, liveliness, and excitability. Each one of these characteristics, according to Eysenck, can be broken down into certain habitual-response patterns that apply to several situations; each of these habitual-response patterns can be broken down further into specific responses within specific situations. This progression from broad, global types down to specific, situation-bound responses is what makes Eysenck's approach a hierarchical theory.

To develop his theory, Eysenck used a kind of steam-shovel approach to personality assessment. He scooped up a massive amount of data from many tests and measures administered to some 10,000 people. Applying factor analysis (Chapter 13, page 525) to the data, he identified (among other things) the typology shown in Figure 14.2. The vertical dimension shows people high in neuroticism at the upper end and people

Figure 14.2



Scores obtained by various neurotic, criminal, and normal groups on Eysenck's neuroticism-stability and introversion-extraversion factors. Dysthymics are moderately depressed people with impaired energy or effectiveness. Hysterics are people with physical symptoms that appear to result from psychological conflict or need. (Modified slightly from Eysenck, H. J. (1967). *The Biological Basis of Personality*. Springfield, IL: Thomas. Courtesy of Charles C Thomas, Publisher, Springfield, Illinois and H. J. Eysenck.)

high in stability at the lower end. The horizonal dimension shows people high in introversion at the extreme left and people high in extraversion at the extreme right. The filled circles inside the figure show the average locations of various groups, based on their test results. For example, American college students scored as very stable and relatively extraverted, whereas college students from England tested out as slightly less stable and less extraverted.

Focusing on extreme cases and "good fits": The "strike-zone" approach Eysenck developed a type theory in which the types are actually personality dimensions, and every individual is scored or rated for his or her position on each dimension. A second general approach to personality types involves specifying certain key characteristics or extreme scores that must be manifest before any individual is said to fit the type. In this approach, people who do not fit the type are simply ignored, and attention is focused on the relatively "pure" cases who fit the "strike zone" for the type in question (see Figure 14.3). This approach is commonly used in diagnosing psychological disorders (see Chapter 15); people must show certain specific personality characteristics to a certain degree before they are typed as having, say, a schizophrenic disorder. The strike-zone approach is also used to identify *Type A* and *Type B* people—two groups who differ in their susceptibility to the major cause of death in the United States, heart disease. For details, see Application 14.1.

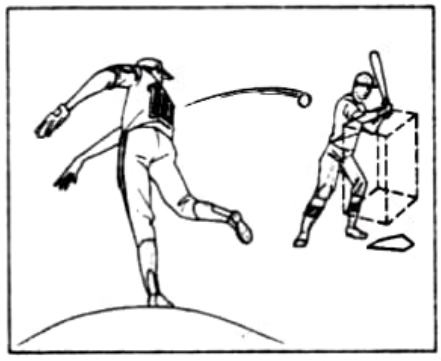


Figure 14.3

In baseball, the strike zone is over the plate and at a specific height—between the batter's knees and just below the shoulders. Anywhere else is the ball zone. Similarly, if people's trait characteristics all fall within a given "type zone," fitting a particular pattern or cluster, we can speak of a psychological *type*.

TRAIT THEORIES

Perhaps when you read about George and Crystal at the beginning of this chapter you did not think of them in terms of types at all. Instead, you may have seen Liddy as willful, determined, flamboyant, and perhaps "inclined to go to extremes to prove himself or make a point." Crystal may have seemed impulsive, strong, and "inclined to make quick decisions and stick by them." Descriptive terms like these represent *traits*—characteristics that lead people to behave in more or less distinctive and consistent ways across situations.



Allport's Theory Years ago, psychologist Gordon Allport counted about 18,000 traitlike terms in the English language—terms that designated "distinctive and personal forms of behavior" (1937, pp. 303–304). These terms, mostly adjectives, describe how people act, think, perceive, and feel. Not all of these terms reflect personality traits, but several thousand of them do. Allport (1961) believed that this rich collection of traitlike terms provided a way of capturing the uniqueness of each individual. He believed that this uniqueness could be described well in terms of the individual's traits, or "personal dispositions," at three levels of generality.

Cardinal traits he defined as those which are so dominant that nearly all of the individual's actions can be traced back to them. These broad, highly influential traits are often called by names drawn from key historical figures. For instance, one person might be described as Christlike, another as Machiavellian, and still another, in recent years, as Kennedyesque or Nixonian. Each term describes a trait so broad and so deep in its impact that it overshadows the influence of other traits in the same individual. Allport believed that most people have *no* true cardinal traits

but that when someone *does* have a cardinal trait it shows itself in virtually all of that person's behavior.

For most people, who are without a cardinal trait, central traits

Application 14.1

PERSONALITY TYPES AND HEART DISEASE

We are inclined to think of heart attacks as a purely medical problem although we all recognize their connection with lifestyle factors such as smoking obesity, and inactivity. During recent years, however, a connection with personality type has also emerged. Two specific behavior-pattern types are now known to be associated with increased and decreased likelihood of coronary-artery disease. Indeed, these behavior-pattern types are apparently better at predicting heart disease than are elaborate medical diagnostic methods such as the examination of a person's blood cholesterol level.

Type A persons are hard-driving and competitive. They live under constant pressure, largely of their own making. They seek recognition and advancement and take on multiple activities with deadlines to meet. Much of the time they may function well as alert, competent, efficient people who get things done. When put under stressful conditions they cannot control, however, they are likely to become hostile, impatient, anxious, and disorganized. They may fume at a slow elevator or a poorly informed salesperson who interferes with their tight schedule, for example.

Type B persons are quite the opposite. They are easy going, noncompetitive, placid, unflappable. They weather stress easily. In some ways, they are like the tortoise in the tale of the tortoise and the hare. Type B's may be a little dull, but they are likely to live longer than the harelike Type A's.

Type A's can be distinguished from Type B's by their answers to personality questionnaires and also by their behavior. Given a task to do, Type A's tend to perform near their maximum capacity no matter what the situation calls for. They work hard at arithmetic problems whether or not a deadline is imposed; Type B's work harder when given a deadline. On a treadmill test (continuous walking on a motorized treadmill), Type A's expend more energy and use a greater proportion of their oxygen capacity than do Type B's, yet they rate their fatigue as less severe. If asked to judge when a minute has elapsed, Type A's judge the periods as significantly shorter than do Type B's. In other words, Type A's show a push toward achievement, a suppression of the cost (fatigue) to themselves, and impatience with delay.

It is particularly interesting, however, that when placed in long-lasting stressful situations over which they have little control, Type A's tend to give up. They show a kind of helplessness (Chapter 15, page 650) and become less responsive and less effective than Type B's. At first, they struggle to control the situation; but when they fail to do so, they stop coping.

There is, of course, a rational explanation for the link between personality type and heart disease. It probably lies in the chemical substances released by the autonomic nervous system (Chapter 8, page 315) in response to stressors. There is some evidence that victims of heart attacks have different reactions to stressors. Either their blood-chemistry response is different or the *timing* of their response is different. There is evidence that their responses are delayed and protracted rather than immediate. All this suggests that programs to help Type A patients cope more constructively with stressors might be helpful. Indeed, a number of psychologists are pursuing that very idea, apparently with some success.

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become crucial. *Central traits* he described as characterizing an individual's behavior to some extent but not in such a complete way as cardinal traits. Allport described central traits as those that might be mentioned in a careful letter of recommendation or checked off on a rating scale where the rater is asked to select the outstanding characteristics of an individual. It would be rare, he wrote, for an individual to have more than 10 or 12 such central traits.

Finally, the least generalized characteristics of the person he labeled *secondary traits*. These are traits such as "likes chocolate" or "prefers foreign cars"—traits that are influential but only within a narrow range of situations. Allport recommended that cardinal, central, and secondary traits be used to assemble what he called psychological life histories and that information about these traits come from materials produced by the individuals themselves—materials such as letters, diaries, or personal journals.

In recommending that people be described in terms of the traits that capture their uniqueness, Allport was subscribing to the *idiographic approach*. This approach involves the psychological study of the individual case; it entails efforts to understand, explain, and sometimes predict an individual's behavior in various situations. A contrasting approach, which Allport also valued (though somewhat less), is called *nomothetic*, or *dimensional*; this approach is aimed at the discovery of personality principles that apply to people in general. The idiographic approach involves a search for consistencies within particular individuals; the dimensional approach involves a search for consistencies and general principles that apply *across* individuals.

Single-Trait Research Allport's approach involved multiple traits, but not all trait theorists and researchers cast such a broad net. Some try to focus carefully on one single trait. For example, many have studied *locus of control*—the degree to which we believe that we cause, or control, the events in our lives. If we believe that we are the cause of most events, we have a highly internal locus of control; if we believe that most events are caused by luck, fate, or powerful others, we have a highly external locus of control. Julian Rotter (1966) developed a questionnaire to measure internal versus external locus of control. It required people to choose between pairs of alternatives such as the following:

1*a*. Students who try their best should have little difficulty making good grades in most classes.
b. Grades in most classes are strongly influenced by teacher bias and other factors beyond the student's control.
2*a*. I am the master of my fate.
b. Most of the things that happen to me in life are caused by forces more powerful than I.

People who are in life circumstances that reduce their "control over reinforcement"—for example, minority groups or new army recruits—tend to choose external responses such as 1*b* and 2*b* more often than do people in more powerful circumstances (see Figure 14.4). Moreover, people who score as highly internal on this questionnaire often seek out learning experiences relevent to their life circumstances. For example,



Figure 14.4

Which of these two people is likely to have a more internally perceived locus of control? (Burke Uzzle/Woodfin Camp & Associates.)

internal patients with life-threatening illnesses have been found more likely than external patients to seek out information about their illnesses and to look for ways of treating it. As research on locus of control illustrates, we can learn useful things about people by focusing on a single trait.

QUESTIONS ABOUT TYPE AND TRAIT THEORIES

Type and trait approaches to personality are close enough to our everyday way of thinking about people as to have a certain ring of truth. Yet truth can be elusive, especially when our quarry is as complex as the human personality. Although type and trait theories have enhanced our understanding of human behavior, critics have questioned whether these approaches give us a complete and accurate picture of personality. Their concerns have fallen into two broad areas: questions about methodology and a philosophical question.

Methodological Questions One of the most basic questions about methodology concerns reliability (see Chapter 13, page 518). Generally, reliability in the form of agreement among observers is not a major problem, provided the observers are given careful training and clear rules for judging trait (or type) dimensions. This is true whether we are speaking of well-developed schemes for observing behavior directly or well-developed tests that yield scores on various traits. Problems of interjudge reliability do arise, however, when the procedures for inferring traits are not clear or when very subjective judgments are involved.

A second question concerns the validity of trait assessments—that is, whether the assessments mean what they are supposed to mean. We know from experience that people may give answers on tests and questionnaires to present themselves in a certain light. Some may try to make themselves "look good." Others may try to make themselves look trou-

bled in order to get help from others. To the degree that such response sets (for example, social desirability or help seeking) influence the scores people receive on trait or type measures, the measures are not doing a valid job of assessing traits and types. Fortunately, some of our measures (for example, the MMPI discussed in Chapter 13) have built-in ways of assessing such response sets.

A third methodological question concerns type and trait consistency. Types and traits are generally thought of as sources of stability in our behavior that lead us to behave in consistent ways across different situations. Yet many research findings over the past 60 years suggest otherwise: People who are honest in one situation are often not so honest in another; people who are calm in one situation may be very anxious in another, and so forth. Findings like these have led some to conclude that type and trait theories may be a bit misleading—that they may depict more consistency in human behavior than really exists. This line of thinking has helped to promote *situationism*—the view that behavior is more a product of the particular situation we find ourselves in than a product of enduring "person" characteristics like traits or types. We will consider the person-versus-situation debate later in this chapter (pages 606–608).

A Philosophical Question One question about trait and type approaches is purely philosophical in nature: Is it really adequate to think of our personality as the sum of our traits or as the particular type we fit? Some argue that personality is much more—that it is an active, dynamic interplay of motives, thoughts, and feelings and that it is best described not as a set of traits or types but as a set of processes by which people cope with life. We turn now to some theories that present this dynamic, process-oriented view of personality.

Dynamic Personality Theories

Before Gordon Allport became a famous trait theorist, he visited an already-famous theorist of a different kind: Sigmund Freud. At the ripe old age of 22, and fresh out of college, Allport wrote a note to Freud asking if he might see the famous psychoanalyst when he, Allport, visited Vienna. Freud very kindly replied with an invitation to visit. Young Gordon was ushered into "the famous red burlap room" which had "pictures of dreams on the walls"; then he was summoned to Freud's inner office. Here, in Allport's words, is what happened next:

> He did not speak to me but sat in expectant silence for me to state my mission. I was not prepared for silence and had to think fast to find a suitable conversational gambit. I told him of an episode on the tram car on my way to his office. A small boy about four years of age had displayed a conspicuous dirt phobia. He kept saying to his mother, "I don't want to sit there . . . don't let that dirty man sit beside me." To him everything was *schmutzig* (filthy). His mother was a well-starched Hausfrau, so dominant and purposive looking that I thought the cause and effect apparent.
>
> When I finished my story Freud fixed his kindly therapeutic eyes upon me and said, "And was that little boy you?"
> (Allport, 1968, p. 383)

The little boy, in fact, was *not* Gordon Allport. Allport's story was, in fact, precisely what he presented it to be: a description of an actual episode on an actual tramcar, involving an actual other person. But Freud's attempt to probe for a deeper meaning reveals an important difference between the type and trait theories of personality that we have just discussed and the dynamic theories to which we now turn our attention. *Type and trait approaches* involve a search for separable components of personality and for the ways those components fit together to form a personality structure. *Dynamic approaches* involve a search for the processes by which needs, motives, and impulses—often hidden from view—interact to produce the individual's behavior. Freud's question to Allport revealed just such a search—that is, a search for any less-than-apparent needs, motives, or impulses which might have led Allport to tell that particular story at that particular time.

Freud's question also reveals a tendency for which dynamic theorists are often criticized: a tendency to overinterpret—that is, to attribute deeper meaning to behavior than the behavior really warrants. A related criticism is that dynamic theorists often make claims about personality processes without testing them scientifically. Both criticisms are often applied to Freud's theory, which remains in some respects one of the least substantiated of the major personality theories. However, Freud's is also the most influential and the most comprehensive theory of personality. It has had a major impact on psychology, sociology, anthropology, and literature—indeed, on western thought in general. Currently its influence and popularity have faded somewhat among some psychologists, partly because there is so little hard, scientific evidence to support the theory. However, recent surveys of psychotherapists (Chapter 16) reveal that more of them subscribe to dynamic theories, including Freud's, than to any other approach. Certainly there is much about Freud's theory that is still widely accepted, and no examination of personality theories would be complete without a careful look at his monumental contribution.



FREUD'S PSYCHOANALYTIC THEORY

We begin our study of Freud's approach with a look at the raw material he used to shape his theory: the thoughts and recollections of his patients. Here is a sample from the analysis of "Frau Emmy von N.," a patient Freud began treating in 1889.

> "When I was a child, it often happened that out of naughtiness I refused to eat my meat at dinner. My mother was very severe about this and . . . I was obliged two hours later to eat the meat, which had been left standing on the same plate. The meat was quite cold by then and the fat was set so hard" (she showed her disgust). . . . "Whenever I sit down to a meal I see the plates before me with the cold meat and fat on them. . . ."
>
> "Many years later, I lived with my brother who was an officer and had that horrible disease [venereal disease]. I knew that it was contagious and was terribly afraid of making a mistake and picking up his knife and fork" (she shuddered) ". . . soon after that, I nursed my other brother when he had consumption [tuberculosis] so badly. We sat by the side of his bed and the spittoon always stood on the table, open" (she shuddered again) ". . . and he had a

habit of spitting across the plates into the spittoon. This always made me feel so sick, but I couldn't show it for fear of hurting his feelings."

(Breuer & Freud, 1893–1895/1955, p. 82)

As "Frau Emmy's" recollections spilled out, Freud's mind was churning, piecing together a complex psychological puzzle. Frau Emmy suffered from anorexia (see Chapter 12, page 480)—she avoided food and drink. Emmy also stuttered and had periodic tics—seemingly uncontrollable facial contortions—and bursts of inappropriate speech. For example, in the middle of a pleasant conversation, Frau Emmy's face would sometimes twist into a grimace of disgust, and she would say, "Keep still! Don't say anything! Don't touch me!" To understand these "symptoms" and their origin, Freud tried in whatever ways he could to unravel the complex fabric of Frau Emmy's personality. Gradually, it occurred to Freud that Emmy's failure to eat and her symptoms of disgust and revulsion might all relate to deeply buried memories—memories like those we quoted above—in which food became disgusting. Unraveling current problems meant digging such memories out of the unconscious.

For this troubled woman, and for most of Freud's other patients, the unraveling and digging process involved reconstructing and even reliving the past—often going as far back as early childhood. Freud's methods evolved over time. He favored hypnosis early in his career (Frau Emmy's recollections, above, were brought out under hypnosis); but he moved gradually toward free association, in which the patient said *everything* that came to mind, no matter how silly, illogical, or even forbidden it might seem. Applying such techniques to a steady stream of patients, Freud pieced together a picture of personality that shook the western world and shaped much of the field of psychology for decades.

Freud's psychoanalysis (Freud, 1920, 1938, 1940/1949) became both a theory of personality and a method of psychotherapy (see Chapter 16, page 681). *Psychoanalytic theory* has three major parts: (1) a theory of the structure of personality, in which the ego, id, and superego are the principal parts; (2) a theory of personality dynamics, in which conscious and unconscious motivation and ego-defense mechanisms play a major role; and (3) a theory of psychosexual development, in which different motives and body regions influence the child at different stages of growth, with effects persisting in the form of adult personality traits.

Personality Structure: Id, Ego, and Superego Freud constructed a model of personality with three interlocking parts: the id, the ego, and the superego.

The *id*, the most primitive part, can be thought of as a sort of storehouse of biologically based urges: the urge to eat, drink, eliminate, and, especially, to be sexually stimulated. The sexual energy that underlies these urges is called the *libido*. The id operates according to what Freud called the pleasure principle. That is, left to itself, the id would satisfy its fundamental urges immediately and reflexively as they arose, without regard to rules, the realities of life, or morals of any kind.

The id, however, is usually bridled and managed by the *ego*. The ego consists of elaborate ways of behaving and thinking which constitute the "executive function" of the person. The ego delays satisfying id

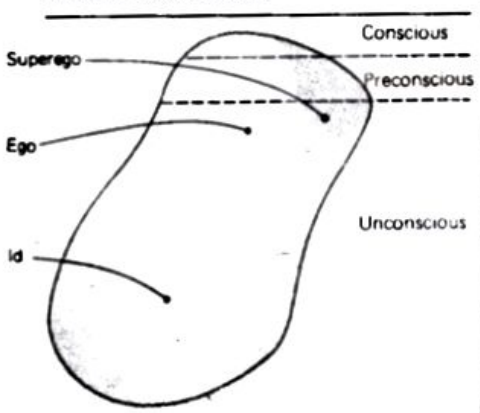
motives and channels behavior into more socially acceptable outlets. It keeps a person working for a living, getting along with people, and generally adjusting to the realities of life. Indeed, Freud characterized the ego as working "in the service of the reality principle." That is, the ego tries to satisfy the id's urge for pleasure but only in realistic ways that take account of what is possible in the real world. The ongoing tension between the insistent urges of the id and the constraints of reality helps the ego develop more and more sophisticated thinking skills.

The *superego* corresponds closely to what we commonly call the conscience. It consists mainly of prohibitions learned from parents and other authorities. The superego may condemn as "wrong" certain things which the ego would otherwise do to satisfy the id. But the superego is not all fire and brimstone. Its conscience-like proddings are also guided by what Freud called the ego ideal, a set of positive values and moral ideals that are pursued because they are believed to be worthy.

Personality Dynamics and Levels of Consciousness Freud did not intend to divide personality into three separate compartments but rather to convey a lively, ongoing interplay among the id, the ego, and the superego. In this interplay, he saw the ego acting as a sort of mediator between the id—with its blind demands for instant gratification—and the superego—with its rigid, often irrational rules, prohibitions, and ideals. The ego's task of satisfying both id and superego requires a somewhat risky balancing act. For example, if the ego yields to the id's desire for something that is morally forbidden, the superego may "punish" the ego with feelings of guilt. Thus the ego's task often involves finding a compromise between the instinctual gratification sought by the id and the strict rule-following sought by the superego. Freud's general notion that our behavior is influenced by biological drives (*id*), social rules (*superego*), and mediating thought processes (*ego*) may not seem farfetched. However, his heavy emphasis on the primitive, sexual nature of human drives and energy (*libido*) helped make his theory very controversial.

Figure 14.5

One can imagine the components of personality, in psychoanalytic theory, something like this. Note how important Freud thought the id to be (its size relative to the other components) and how large a part of the personality remains unconscious.



Less controversial but equally novel was Freud's notion of unconscious processes. He used this concept to explain why people often act in ways that seem irrational. Freud proposed three levels of consciousness, or awareness: the *conscious*, the *preconscious*, and the *unconscious*. At the conscious level, we are aware of certain things around us and of certain thoughts. At the preconscious level are memories or thoughts that are easily available with a moment's reflection—for example, what we had for breakfast, or our parents' first names. In contrast, the unconscious contains memories, thoughts, and motives (Chapter 7) which we cannot easily call up. All of the id is unconscious; the ego and superego include material at all three levels of consciousness (see Figure 14.5).

Why do some ideas and feelings become unconscious? There are several answers, but one that became a centerpiece of Freud's theory is summed up in the word *repression* (discussed further on page 588). We *repress*, or banish from consciousness, ideas, memories, feelings, or motives that are especially disturbing, forbidden, or otherwise unacceptable to us. The process of repression is itself unconscious and automatic. We do not *choose* to repress an idea or impulse—it just happens, whenever the idea or impulse is so painful and anxiety-arousing that we must escape

from it. In such cases, our anxiety triggers repression, and the unacceptable material is buried in the unconscious.

The material we repress is usually something that clashes painfully with our ethical standards or self-image. For example, one of Freud's patients, Fräulein Elisabeth von R., had experienced what she knew was an unacceptable desire for her sister's husband. Her sister had grown very ill, and one day Fräulein Elisabeth arrived at the sickbed to find that her sister had died. Freud described Elisabeth's reaction.

> At that moment of dreadful certainty that her beloved sister was dead . . . another thought had shot through Elisabeth's mind, and now forced itself irresistibly upon her once more, like a flash of lightning in the dark: "Now he is free again and I can be his wife." (Breuer & Freud, 1893–1895/1955, p. 156)

Here was a shocking idea, totally foreign to Elisabeth's moral standards and to her view of herself as an ethical person. Not only had she desired her sister's husband while her sister lay ill and defenseless, but even at the moment of her sister's death her first thought had been one of selfish joy—that she might profit from the death by gaining a husband. This whole set of ideas and motives conflicted so painfully with Elisabeth's moral standards and her self-image that it was repressed and exiled to her unconscious. She never thought about it again, at least not until Freud brought the memories back through psychoanalysis.

Why bring the memories back, you might ask. Why not leave them safely tucked away in the unconscious? The answer, according to Freud's theory, is that repressed material does not remain *safely* tucked away. Instead, it continues to operate underground, often converting the repressed conflicts into *neurosis*—disturbed behavior involving anxiety or defenses against anxiety. Neurotic symptoms often bear a *symbolic* relationship to the repressed material that is causing them. For example, Fräulein Elisabeth was first attracted to her brother-in-law when the two went for a long walk at a summer resort. After her sister's death, she developed severe pains in her legs and was eventually unable to walk or even to stand up. It was as if she was being punished for the feelings she developed during a walk by developing symptoms that would prevent her from walking. Freud believed that for Elisabeth and other people as well, repressed material is not dormant but active—in unconscious, often painful ways.

Unconscious processes also figured prominently in Freud's ideas about dreams and "accidents." (See Chapter 5, page 207.) He saw dreams as disguised manifestations of id motives, and he described dreams as "the royal road to the unconscious." In everyday life, he said, unconscious thoughts and forbidden impulses are revealed by accidents and by slips of the tongue or the pen. For example, Freud told of a certain Social-Democratic newspaper that reported on activities of "the Clown Prince" instead of "the Crown Prince." The paper ran an apology the next day, correcting the misprint so that it read "the Crow Prince." In another example, Freud described a letter in which a patient attributed his nervousness to a cold wave that had killed off the crops. But instead of writing "cold wave," the patient wrote "cold wife." This "slip," Freud

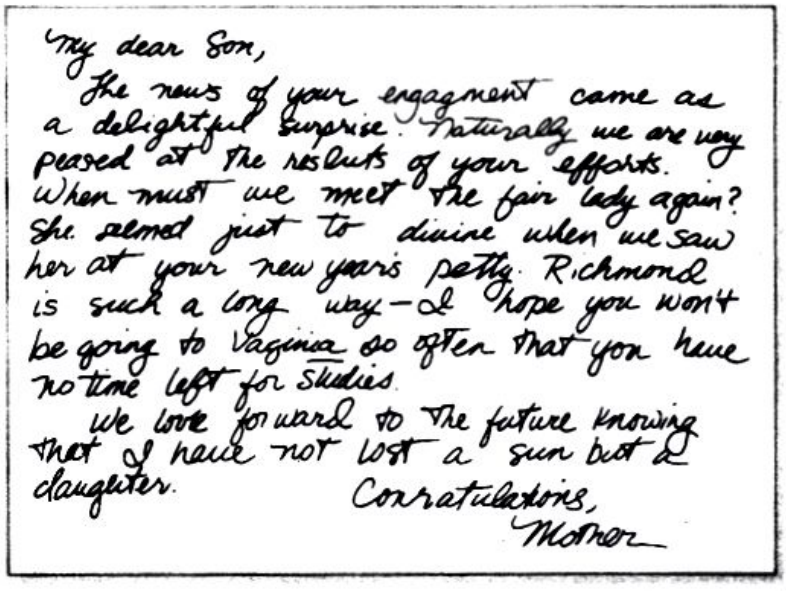
said, grew out of the patient's unconscious anger toward his wife for her "marital frigidity." Even today, slips in letters may point to unconscious motives; take a look at Figure 14.6 and see how many revealing slips you can find.

Psychosexual Stages of Development Freud put heavy emphasis on biological development in general and on sexual development in particular. In his theory of child development, Freud described a succession of stages revolving around body zones. Freud's idea was that from birth on, we have an innate tendency to seek pleasure, especially through physical stimulation and particularly through stimulation of parts of the body that are sensitive to touch: the mouth, the anus, and the genitals. It was no accident that Freud referred to these parts of the body as *erogenous zones*. He saw the pleasure derived from touching and rubbing these sensitive zones as *erotic*, or sexual, for infants as well as adults. Flip back to page 410, and review the quotation from Freud. There he described the baby's sucking as a sensual activity that leads to something very much like an orgasm. For babies, the most sensitive erogenous zone is the mouth. With advancing age, however, other body zones become especially sensitive to stimulation. With each of these shifts in the focus of sexual stimulation come parallel shifts in the dominant psychological issues faced by the person. Thus Freud called each step in this process a *psychosexual stage*.

Freud believed that if a child's needs at one of the psychosexual stages were either unsatisfied or oversatisfied, *fixation* would take place. That is, the child would show continued attachment to an old stage even after moving on to a new one. As a result, behavior patterns—and problems—from the fixated stage would persist, often into adulthood. By observing an adult's behavior carefully, one could recognize the psychosexual stage in childhood at which the adult had fixated. We turn now to a description of the psychosexual stages and some of their implications for adult personality.

Figure 14.6

An exercise in the detection of Freudian slips—in this case, slips of the pen. A woman writes to her son after receiving the news of his engagement to a woman from Richmond, Virginia. In commenting on this "delightful surprise," she may be telling her son more of her true feelings than she realizes. How many slips of the pen can you spot? (From *Psychology and Life* (10th ed.) By P. G. Zimbardo. Copyright © 1979 by Scott, Foresman and Company. Reprinted by permission.)



My dear Son,

The news of your engagment came as a delightful surprise. Naturally we are very pleased at the results of your efforts. When must we meet the fair lady again? She seemed just to divine when we saw her at your new years party. Richmond is such a long way—I hope you won't be going to Virginia so often that you have no time left for Studies.

We look forward to the future knowing that I have not lost a son but a daughter.

Corratulations,

Mother

In the *oral stage* (birth to about age 1), according to Freud's theory, the infant obtains sensual pleasure first by sucking and later by biting. Feeding and contact with the mother, mouthing new objects, and even relief of teething pain by biting—all help to make the mouth the focus of pleasure during the first year. A baby given too little opportunity to suck (or too much), or made anxious about it, may acquire an oral fixation which, in adulthood, may foster excessive oral behavior—for example, "taking in" in concrete forms such as smoking or in psychological forms such as dependency. (See Figure 14.7.) Fixation during the oral biting stage may produce a critical, "biting" personality.

The *anal stage* (ages 1–3) occurs when parents are toilet training their children and teaching them to avoid prohibited behavior connected with excretion. At this time the region around the anus becomes highly sensitive to the stimulation of "holding on" and "letting go." In our society, toilet training is ordinarily the child's first encounter with authority and the first time the id must be brought under control of the emerging ego. Psychoanalytic theory holds that the first part of the anal stage involves pleasure from expulsion of feces; the latter part involves pleasure from retention. Freud maintained that fixation at the first substage results in adult characteristics of messiness and disorder; fixation at the latter substage results in excessive compulsiveness, overconformity, and exaggerated self-control.

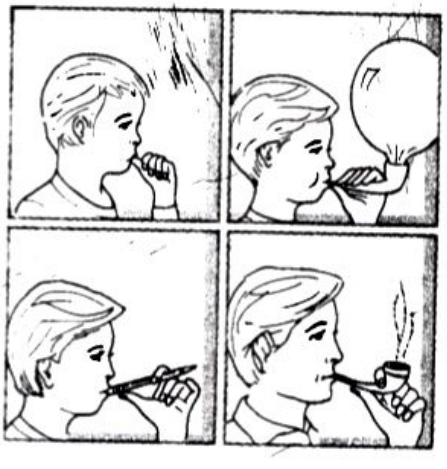
After they master toileting, children grow more interested in their genitals. In fact, Freud believed that genitals become the major focus of sexual excitement in the *phallic stage* (ages 3–5). It is also at this time that he felt children developed sensual feelings toward the parent of the opposite sex. Freud called these thoughts and feelings in boys the *Oedipus complex* after the mythical story of Oedipus, who unknowingly killed his father and married his mother; in girls, he called the thoughts and feelings the *Electra complex* after Agamemnon's daughter, who arranged for her mother to be murdered.

In the phallic stage, the boy's desire for stimulation of his penis is associated with his attachment to his mother, and he comes to desire her in a sexual, or at least a sensual, way. Eventually, the boy becomes fearful that his father knows of these feelings and will be furious. He fears retaliation by his father, retaliation aimed at the "source of the problem"—the boy's penis. In other words, the boy fears castration. To cope with the severe anxiety that this fear brings on, the boy begins to construct psychic defenses, especially the defense of *identification*—the boy tries to become like his father. Anxiety is reduced because a threatening father would not be likely to harm someone just like him—a "chip off the old block." In the process of identifying with his father, the boy not only takes on his father's behavior patterns but also his father's ideas of right and wrong. Thus, it is through identification in the phallic stage that the boy's superego begins to form.

For the girl, the sequence begins with an erotic focus on the father. But, in addition, the girl notices that she does not have the sexual organs of her father or brothers and she experiences "penis envy." She suspects that she may actually have been castrated by her mother; this makes her angry, and she comes to resent and devalue her mother. Nonetheless, she eventually identifies with her mother—partly because she suspects that if she takes on her mother's characteristics she will stand a better chance in

Figure 14.7

Changes occur in the normal expression of oral behavior as a person grows older. Fixation at the oral stage may produce an oral syndrome characterized by excessive oral behavior and such traits as dependency and passivity. (After Wattenberg, 1955.)



her own "romantic relationship" with her father. Thus, in spite of her affection for her father and her resentment of her mother, the little girl identifies with her mother, behaving like her and incorporating her values. In this way, the girl develops her superego.

Of course, the specifics of the phallic stage vary from child to child. To illustrate, we have included one of Freud's most famous case histories, "Little Hans," discussed in Application 14.2, was, in Freud's opinion, caught up in the throes of the Oedipus complex.

Freud believed that superego strength—that is, how harsh one's conscience is in punishing misbehavior—depended largely on events of the phallic stage. Very punishing, anxiety-ridden experiences were said to produce a strong, punishing superego.

The *latency period* (6 through puberty) was not considered by Freud to be very important to the development of personality. As the child learns more about the world, sexuality is largely repressed and the ego expands.

Puberty marks the beginning of the *genital stage* (adolescence and beyond), when mature heterosexual interests appear. There are three major sources of sexual arousal during this period: memories and sensations from earlier childhood periods, physical manipulation of genitals and other erogenous zones, and hormonal secretions. Many of the themes and anxieties of earlier stages resurface, but in new and more mature forms. In particular, the targets of sexual arousal now lie outside the tiny circle of self and family. Mature heterosexual relations emerge, with the species-preserving possibility of procreation now very real. The stage is set for esponsible enjoyment of adult sexuality—which was, for Freud, the epitome of healthy development.



JUNG'S ANALYTICAL PSYCHOLOGY

Carl Gustav Jung (pronounced "Yoong") was, at one time, considered by Freud to be his heir apparent. In 1907, Freud wrote to Jung, "I could hope for no one better than yourself . . . to continue and complete my work" (McGuire, 1974, p. 27). Yet the direction that Jung took with his *analytical psychology* (Jung, 1928) differed from psychoanalysis in some very significant ways, and Freud's affection for Jung faded. One of the main differences between the theorists was that Jung thought childhood psychosexual development to be not nearly so important to adult adjustment as Freud did. Jung placed much less emphasis than Freud on sexual and aggressive impulses arising from past conflicts and much more emphasis on people's future-oriented goals, hopes, and plans. The two men also had quite different ideas about the nature of the unconscious. To appreciate Jung's ideas on this subject and others, we need to first consider his background, beginning with his childhood.

From childhood on, Jung was extremely interested in dreams and fantasy. Some of his boyhood dreams even terrified him. Young Carl's minister father was often depressed and prone to doubt his faith, and some of Carl's most powerful dreams involved religious images and symbols. These dreams were often laced with forbidden and sacrilegious overtones—for example, a godlike figure that looked like a giant penis. Such boyhood dreams helped give Jung, the theorist, his lifelong fascination with the content and meaning of dreams and visions in people's lives.

In developing his theory, Jung drew on both his own dream and

Application 14.2

A PSYCHOANALYTIC CASE HISTORY

One of the most famous cases in the history of psychoanalysis is that of "Little Hans," a 5-year-old who revealed many of his perceptions, fantasies, and fears to his physician father, who, in turn, reported them to Sigmund Freud. The case of Little Hans illustrates many features of the phallic stage of development as well as some ideas of psychoanalytic theory about phobias, or irrational fears.

Little Hans, from the age of 3 and on, showed a lively interest in that part of his body which he called his widdler. He had an active sexual curiosity that was particularly directed toward members of his family. For a time, he maintained that "widdlers" were possessed by all animate objects and were the feature which differentiated animate from inanimate objects. Naturally, he surmised that girls and women had widdlers, only small ones. Even when he viewed his newborn sister, the evidence did not dissuade him. His frequent conversations about widdlers revealed his misinterpretations of things he saw. For example, he thought a monkey's tail was a very long widdler. At a railroad station, when he saw an engine discharging steam, he assumed that it was using its widdler. He sometimes fondled his widdler, and when he was 3½ his mother threatened him with castration if he continued this behavior. Some guilt about masturbation dated from that time.

As one would predict from psychoanalytic theory, Han's sexual interest and focus on his widdler began to intensify at about age 4. When he was 4½, for instance, he made it clear that having his knickers unbuttoned and his widdler taken out was quite pleasurable, and he finally acknowledged that there was, indeed, a distinction between male and female genitals. At age 5, Hans awoke one morning in tears after an anxiety dream in which his mother was gone. His anxiety—together with a fantasy about a big giraffe and a crumpled giraffe (interpreted as his father's penis and his mother's genitals)—was thought to express an erotic longing for his mother.

Hans began to fear horses, this fear grew to a point where he would not leave the house (horse-drawn carriages were the major means of transportation in Freud's Vienna). The immediate event which precipitated this phobia was seeing a big, heavy horse fall down. Freud interpreted this to mean that Hans, at that moment, perceived his own wish that his father would fall down (die). Then Hans, a little Oedipus, could take his father's place with his beautiful mother. Another part of the fear derived from the large size of horses (and their widdlers), which Hans unconsciously identified with the great power of his father. He expressed the fear that a horse would come into his room. He also became afraid not only of horses but of carts, furniture vans, and buses. This revealed, to the psychoanalyst, still another aspect of Hans's unconscious fantasies: that the falling-down horse was not only his father but his mother in childbirth (with the boxlike carts and vehicles representing the womb). All these complicated, repressed (and, in psychoanalytic theory, universal) feelings and perceptions were thus incorporated into a single phobia.

It is important to note that Little Hans was basically a straightforward, cheerful child; he experienced normal psychosexual development, marred only by the episode of the phobia, from which he recovered rather promptly. Fourteen years later, a 19-year-old Hans came to see Freud. He had continued to develop well and had survived, without unusual difficulty, the divorce and remarriage of both parents. The problems of his childhood (which by then he could not remember) were used by Freud to illustrate the normal process of psychosexual development—the complex, intense, erotic drama of early childhood.

REFERENCE

Freud, S. (1925). Analysis of a phobia in a five-year-old boy. In *Sigmund Freud: Collected Papers* (Vol. 3) (A. Strachey & J. Strachey, Trans.). London: Hogarth Press. (Original work published 1909)

fantasy material and that of his patients; but he also drew on findings from such experimental techniques as the word-association test, in which people were read a standard array of 100 terms (for example, "head," "to

sin," "to pray," "bride," "to abuse") and instructed to respond to each term "as quickly as possible with the first word that occurs to you." In addition to the content of the test taker's associations, Jung recorded how long it took them to respond and whether certain words led to particular patterns of breathing or even perspiration. Stimulus terms that resulted in long delays, an inability to respond, or certain other key signs were thought by Jung to be parts of what he called *complexes*. He defined a *complex* as a network of ideas bound together by a common emotion or set of feelings. For example, Freud's patient, Frau Emmy (page 576), might be said to have had an eating complex.

In using his word-association test to diagnose complexes, Jung believed he was exploring the unconscious, much as Freud had done with hypnosis and free association. Yet Jung's findings with his own techniques led him away from Freud's ideas in several respects. As we noted above, Freud emphasized sexual impulses and motives as the motor of human behavior—a prime source of the libido's energy. But Jung saw the driving force that fueled the libido as "only a continuous life urge," a striving to live and insure the survival of one's species. In addition, Jung's work led him to emphasize some personality structures whose existence Freud either doubted or considered unimportant.

One such structure was the collective unconscious, which Jung believed to be a part of the unconscious mind that went beyond the personal experiences of the individual. The *collective unconscious*, he said, grows out of the past experiences of the human race. Stored within this collective unconscious are primitive fundamental images, impressions, or predispositions that were common to earlier members of the human race. He called these images, impressions, or predispositions *archetypes*. Archetypes are hard to define. They do not have real concrete content, and they are not quite like memories. They are more like subjective reactions that originated in our ancestors in response to certain universal experiences—such as the recurrent rising and setting of the sun, the presence of males and females, and so forth. We have inherited tendencies toward similar subjective reactions, and these tendencies are our archetypes. Put another way, archetypes are inherited ways of organizing, or reacting to, our experiences with the world. Some of the most common archetypes involve God, rebirth (or resurrection), devil (or the embodiment of evil), wise old person, mother, trickster or magician, hero, *animus* (a male archetype present in females), and *anima* (a female archetype present in males). Because each archetype involves strong emotion, Jung believed that people's emotion-generated behavior could often be explained by identifying the key archetype that was influencing them. He applied this principle to groups as well; for example, the ascendence of the God archetype might have played a role in the Crusades; the ascendence of the hero archetype (or perhaps the devil archetype) in post-World War I Germany might have played a role in Adolf Hitler's rise to power.

In addition to the collective unconscious, Jung credited each individual with a *personal unconscious*, which developed out of any of the individual's conscious experiences that had been repressed (page 578). Psychologically healthy people were said to gradually come into contact with the unconscious parts of their personalities, integrating the unconscious or "shadow" side with their "conscious ego." In this way, all major components of the personality could eventually work in concert to form a

fully realized, purposeful self. This process of harmonizing one's conscious and unconscious components happened in a unique way for each person, he said, and led to a unique pattern of behavior. Jung called this process *individuation*; he saw it as the means by which each of us becomes an individual distinct from others.

Jung's ideas, often mystical and complex, have been difficult to test experimentally. However, the ideas have intrigued and attracted many—as evidenced by the numerous "Jung societies" that have sprung up in North America and Europe. In addition, some of Jung's concepts, such as "the complex," have found their way into everyday use. Other Jungian concepts have spurred important work by later theorists. For example, Jung proposed the concepts of *introversion* and *extraversion*—turning inward toward contemplation or outward toward others. These two concepts were later picked up and revised by H. J. Eysenck, eventually becoming a key part of his personality theory (page 570). Jung's ideas have certainly outlived him, and they continue to provoke interest.

ADLER'S INDIVIDUAL PSYCHOLOGY

The third dynamic theorist we will consider is Alfred Adler. He was a colleague of Freud and Jung, but his theory took a different direction from theirs (Adler, 1929, 1931). To explain Adler's focus, we will begin with a story that he liked to tell.



> . . . three children . . . were taken to the zoo for the first time. As they stood before the lion's cage, one of them shrank behind his mother's skirts and said, "I want to go home." The second child stood where he was, very pale and trembling, and said, "I'm not a bit frightened." The third glared at the lion fiercely and asked his mother, "Shall I spit at it?" The three children really felt inferior, but each expressed his feelings in his own way, consonant with his style of life.
> (Adler, 1931, p. 50)

Adler's story illustrates some key differences between his dynamic theory and the theories of Freud and Jung. First, Adler believed that like the hypothetical children in this story, people are forever struggling to overcome their feelings of inferiority. In fact, in a sharp departure from Freud and Jung, Adler identified this struggle as *the* most basic life urge. Acting on this urge, people strive continually, he said, for "superiority" and "mastery of the external world." When someone fails repeatedly to overcome weaknesses and achieve some mastery or simply places excessive emphasis on some particular inferiority, the result can be what Adler labeled an *inferiority complex*, a term that is now part of our everyday language. Adler also stressed the concepts of *compensation* and *overcompensation*—the pursuit of activities designed to make up for or to overcome inferiority (for example, spitting at the lion in the story above). These concepts had great personal significance for Adler; as a child, he was chronically ill, weak, and intensely afraid of death. He compensated, in part, by his career choice: "I came to choose the occupation of physician in order to overcome death and the fear of death" (Adler, in Ansbacher & Ansbacher, 1956, p. 199).

A second distinctive concept of Adler's that is reflected in the zoo

story is the *style of life*. Each individual creates his or her own personal approach to living; this highly individualized style grows out of the individual's unique sense of his or her own inferiorities and the strategies he or she develops to overcome these inferiorities. Just as each child in the zoo story coped differently with fear of the lion, each of us develops a unique and relatively lasting method of coping with life. This concept, together with the concept of the subjective nature of individual goals, led Adler to label his theory *individual psychology*.

A third contribution of Adler's theory is the close attention he gave to birth order as an influence on personality development. First-borns, said Adler, begin life as the exclusive focus of their parents' attention and then are often abruptly dethroned with the birth of their first siblings. The result may be that the child feels cheated and later becomes an unruly misfit. A more positive possibility is that first-borns may develop a style of life in which they behave in a parentlike way toward their siblings and become particularly responsible adults. Adler described other possibilities for second-born, last-born, and only children. Many of his specific ideas have not been borne out, but his ideas did help stimulate some important research. Thanks to this research (for example, Dunn & Kendrick, 1982; Sutton-Smith & Rosenberg, 1970), we now know that first-borns do have especially intense relationships with their parents and that they tend to be more achievement-oriented and self-controlled than do later-born children. Adult firstborns are also more likely to be listed in *Who's Who*, while later-born children tend to be less conforming, more sociable, and more popular. So Adler's general idea that birth order is related to personality does seem to be correct.

DEFENSE MECHANISMS IN THE DYNAMIC THEORIES

Thus far, we have emphasized disagreements among the dynamic theorists, but there are areas of agreement. One of these is the notion that people use *defense mechanisms* to reduce their anxiety and guilt (A. Freud, 1946). Psychoanalytic theory holds that because the id's unconscious demands are instinctual, infantile, and amoral, they must often be blocked by the ego and superego. Because of this conflict and the persistence of unsatisfied demands, *anxiety* (vague fearfulness) and guilt are aroused. The person then seeks ways to protect the ego from this anxiety and guilt by setting up defenses. Freud described several defense mechanisms by which the ego disguises, redirects, hides, and otherwise copes with the id's urges. Others have been added by the dynamic theorists who followed Freud.

Many psychologists do not agree with Freud's view that defense mechanisms originate in conflicts among the id, ego, and superego. However many do agree that these mechanisms account for some of the ways people cope with their problems. Thus, defense mechanisms—an intellectual bequest from the dynamic theories—are generally accepted as a useful way of looking at how people handle stressful situations and conflicts. (See Chapter 7, page 300.)

Repression In Freud's theory, *repression* is the fundamental technique people use to allay anxiety caused by conflicts. As we noted on page 578, repression is an active mental process by which a person "forgets" by "pushing down" into the unconscious any thoughts that arouse anxiety. In other words, as a therapist and theorist named R. D. Laing put it, we forget and then forget that we forgot. In terms of psychic energy, repres-

sion is an expensive defense mechanism. The unconscious memories or urges continue to seek expression and may emerge in the form of "accidents," "slips," or neurotic symptoms (page 579).

Reaction Formation Reversal of motives is another method by which people attempt to cope with conflict. A motive that would arouse unbearable anxiety if it were recognized is converted into its opposite. For example, a person who was reared to believe that sex is evil and "dirty" may be painfully anxious every time sexual feelings surge to the surface. The person may defend against that anxiety by dressing and behaving in very puritanical ways and perhaps even by joining organized crusades against sex in the media. The implicit principle seems to be that "the best defense is a good offense." Similarly, if people are *too* modest, *too* solicitous, *too* affectionate, or *too* strident in their crusades against an "evil" such as alcoholism, homosexuality, or child abuse, it is possible that they are unconsciously harboring the opposite feelings. Thus disguised, the unwanted motives can be controlled. A quotation from Shakespeare captures the idea of reaction formation: "The lady doth protest too much methinks."

Projection Blaming others, or *projection*, is a way of coping with one's unwanted motives by shifting them on to someone else. The anxiety arising from the internal conflict can then be lessened and the problem dealt with as though it were in the external world. For example, an insecure student may have a strong desire to cheat on an examination, but his conscience will not allow him even to consider such a thing. He may then suspect that other students are cheating when they in fact are not. Carried to the extreme, projection is the mark of a behavior disorder known as paranoia (Chapter 15, page 644). People with this disorder may project their own unacceptable hostile feelings about others into a whole system of thinking in which they feel that others are out to get them.

Rationalization This defense mechanism substitutes an acceptable conscious motive for an unacceptable unconscious one. Put another way, we "make excuses," giving a reason different from the real one for what we are doing. Rationalization is not lying; we believe our explanations. Examples range from the innocent to the serious. (See Figure 14.8.) The long lines at the movies during finals weeks are populated by students



Figure 14.8

Rationalization is an everyday occurrence. (Copyright 1977 by King Features Syndicate, Inc.)

who "need to relax" to do a good job on their tests. A tense father who strikes a rambunctious child may rationalize that he is acting for the child's good. Aesop's fable of the fox and the sour grapes is another example of rationalization: Something we cannot get becomes something we did not want anyway. Rationalization is a common mechanism we all use to bolster our self-esteem when we have done something foolish. If overused, however, it can prevent us from coping with a situation head-on. For example, a person with an unconscious fear of intimate relationships may find a succession of potential mates unacceptable for different reasons and, as a result, spend the rest of life alone.

Intellectualization Related to rationalization is *intellectualization*, another defense mechanism which involves reasoning. In intellectualization, however, the intensity of the anxiety is reduced by a retreat into detached, unemotional, abstract language. Professionals who deal with troubled people may intellectualize in order to remain helpful without being overwhelmed by sympathetic involvement. For example, a nurse may describe in an intellectual fashion an encounter with a dying or angry patient. Some adolescents discuss their new experiences with sex and independence on an abstract and impersonal plane. Temporarily separating emotional and cognitive components sometimes helps the individual to deal with parts of an experience when the whole is too much to handle.

Displacement In *displacement*, the motive remains unaltered but the person substitutes a different goal object for the original one. Often the motive is aggression that for some reason, the person cannot vent on the source of the anger. A person who is angry with his or her boss but cannot show it for fear of being fired may come home, bawl out the children, and kick the dog. When a new baby is the center of attention, an older child may become jealous; prevented from harming the baby, the child demolishes a doll. Thus by displacing aggression, the child finds a substitute outlet.

Regression In the face of a threat, one may retreat to an earlier pattern of adaptation, possibly a childish or primitive one. This is called *regression*. Faced with the upsetting arrival of a new baby or going to school for the first time, a 5-year-old may have toilet accidents, revert to "baby talk," demand cuddling, or suck her thumb. (See Figure 14.9.) Adults, too, sometimes revert, when in stress-producing situations, to childish episodes of exaggerated dependency. Such behavior may ward off anxiety by focusing attention on earlier ways of achieving tranquility.

Sublimation For Freud, sublimation was the highest level of ego defense. It consists of a redirection of sexual impulses to socially valued activities and goals. For example, a writer may divert some of his or her libido from sexual activity to the creation of a poem or novel, thus indirectly satisfying the same drives. Freud believed that much of our cultural heritage—literature, music, art—is the product of sublimation. He also believed that satisfactory sublimation could only be achieved by an individual whose sexual impulses were being at least partly gratified and whose ego was healthy and mature.

LEARNING is a key process—some would say *the* key process—in human behavior; it pervades everything we do and think. (But see Chapter 2, pages 39–45.) It plays a central role in the language we speak, our customs, our attitudes and beliefs, our goals, our personality traits (both adaptive and maladaptive), and even our perceptions. Having mastered its fundamental principles, you will be in a position to understand how learning plays its important role in many of the psychological processes described later in the text.

Learning can be defined as any relatively permanent change in behavior that occurs as a result of practice or experience. This definition has three important elements.

1. *Learning* is a change in behavior, for better or worse.

2. It is a change that takes place through practice or experience; changes due to growth or maturation are not learning. This part of the definition distinguishes learning from innately controlled species-typical behavior of the sort that Chapter 2 (page 39) describes.

3. Before it can be called *learning*, the change must be relatively permanent; it must last a fairly long time. Exactly how long cannot be specified, but we usually think of learned changes in behavior as lasting for days, months, or years, unlike the temporary behavioral effects of factors such as alertness or fatigue.

Think about yourself and learning. You are reading this book; you will learn something about psychology from it, but book learning is only a small part of the learning you have done in your life. When you got up this morning, you dressed in a certain way, you ate certain things for breakfast (and did not eat others), and you began to think about the day ahead of you. Thinking invoked your attitudes about other people or events: perhaps it caused you to worry about something. How you dressed, what you ate, what you thought about, how you evaluated other people and events, what you worried about—are all rooted in your past experience, or *past learning*. If we could understand the learning process and apply our general understanding of it to a particular person's life, we would go a long way toward explaining many of the things that person does. If we could understand some of the principles of learning, we would have a better idea of how to change behavior when (as in child rearing and psychotherapy) we want to change it. (See Chapters 11 and 16.)

Classical Conditioning

Classical conditioning gets its name from the fact that it is the kind of learning situation that existed in the early "classical" experiments of Ivan P. Pavlov (1849–1936). In the late 1890s, this famous Russian physiologist

began to establish many of the basic principles of this form of conditioning. Classical conditioning is also sometimes called *respondent conditioning* or *Pavlovian conditioning*.

ACQUISITION, OR LEARNING, OF CONDITIONED RESPONSES

What is classical conditioning? Suppose we start with some examples. Here is one from Pavlov's laboratory:

> Pavlov designed an apparatus, shown in Figure 4.1, that could measure how much a dog's mouth waters in response to food or other things in its environment. At the beginning of his experiment, Pavlov noted that no saliva flowed when he rang a bell. He then trained the dog by sounding the bell and, shortly afterward, presenting food. After the sound of the bell had been paired with food a few times, he tested the effects of the training by measuring the amount of saliva that flowed when he rang the bell and did *not* present food. He found that some saliva was produced in response to the bell alone. He then resumed the training—paired presentations of bell and food—a few more times and then tested again with the bell alone. As training continued, the amount of saliva on tests with the bell alone increased (up to a point, of course). Thus, after training, the dog's mouth watered—salivated—whenever the bell was sounded. This is what was learned; it is the *conditioned response*.

Here is another example from an experiment done by one of the authors many years ago (King et al., 1961).

> Student volunteers were the subjects in this experiment. Each student sat in a booth in which a brief jet of air could be puffed at his or her right eye. The response to this puff was a sharp blink of the eyes. One-half second before each puff, a dim spot of light came on. Tests showed that at the beginning of the experiment, the students

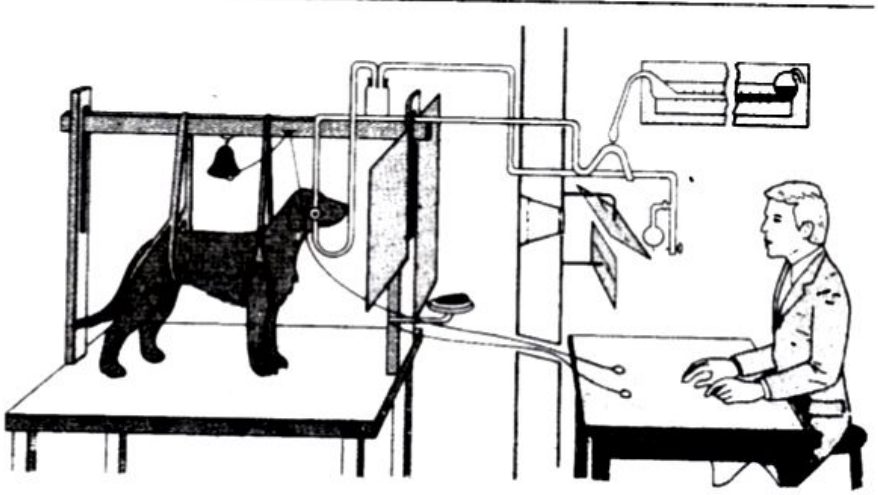


Figure 4.1

Pavlov's apparatus for studying the conditioned salivary (drooling) response.

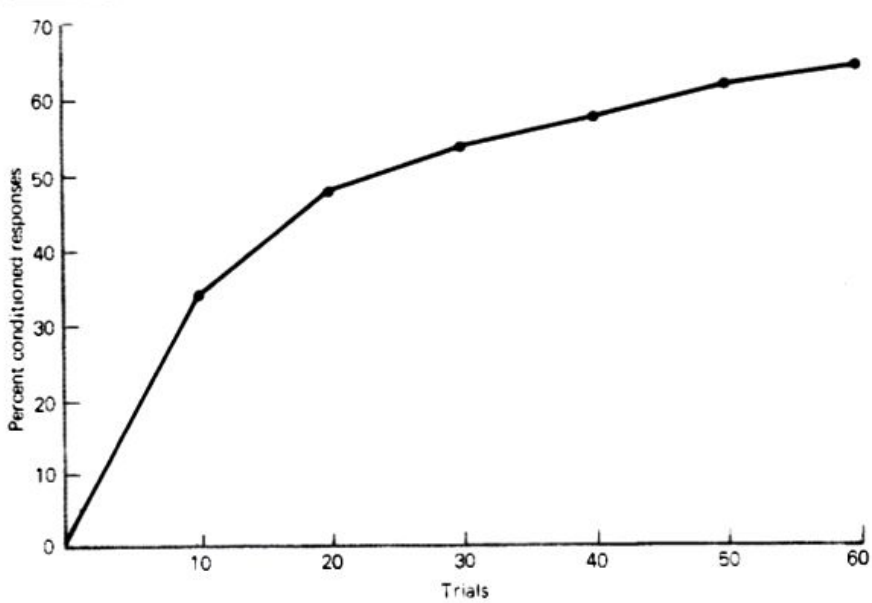
did not blink in response to the light. The light and puff were paired—light followed by puff—a number of times. Soon the students began to blink when the light came on *before* the puff. The number of blinks to the light increased steadily as more and more pairings were given. Figure 4.2 shows how the blinking to the light increased during the experiment. (Each point on the graph shows the percentage of blinks to the light during 10 pairings of light and puff.) The students had learned to blink when the light came on; this is the conditioned response in this experiment.

If you want a third example, look back at the opening pages of this chapter. What is the classically conditioned response in the therapy for bed-wetting?

What do these examples of classical conditioning have in common? In other words, what are the general characteristics of situations in which conditioned responses are acquired, or *learned*? In classical conditioning, two stimuli are presented to the learner. (The term *stimulus* comes from the Latin word for "goad," or "prod"; and thus, in psychology, the term is sometimes used to refer to events which evoke, or call forth, a response—the individual is "goaded into action." A more general meaning of the term *stimulus* is anything in the environment that can be detected by the senses.) One of the stimuli in classical conditioning is called the *conditioned stimulus (CS)*. It is also known as a neutral stimulus because except for an alerting, or attentional, response the first few times it is presented, it does not evoke a specific response. Almost any stimulus which is detectable can serve as a CS. The bell and the light were the CSs in the two examples that opened this section. What was the CS in the behavior-therapy example at the beginning of the chapter? The other stimulus is known as the *unconditioned stimulus (US)*. This stimulus consistently evokes a response or is reliably followed by one. The response that reliably follows the unconditioned stimulus is known as the *unconditioned*

Figure 4.2

A learning, or acquisition, curve for an experiment on the classical conditioning of eye blinking. (From King et al., 1961.)



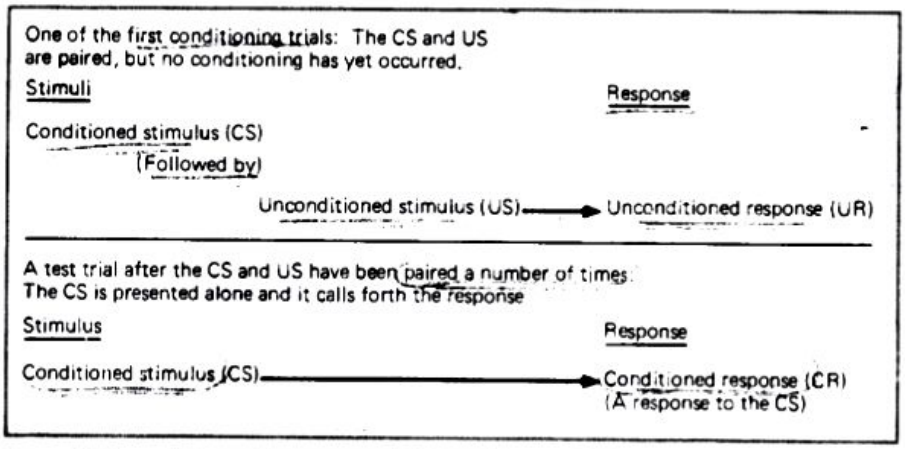


Figure 4.3

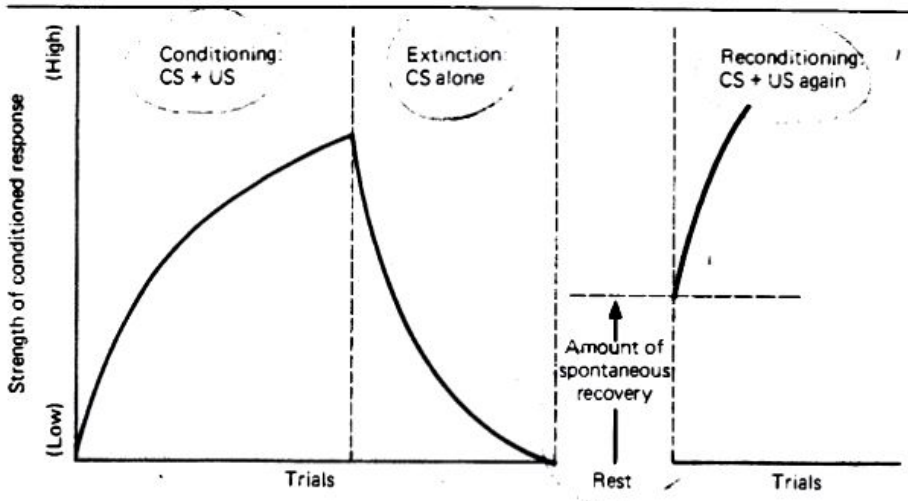
A chart of events in classical conditioning.

response (UR). What were the USs and the URs in the examples? The two stimuli—the CS and the US—are paired in classical conditioning so that the conditioned stimulus comes a short time—say from ½ second to several seconds—*before* the unconditioned stimulus is presented. After the stimuli have been paired a number of times (each pairing is called a trial), presentation of the originally neutral conditioned stimulus evokes a response. This response is what is learned in classical conditioning; it is termed the *conditioned response (CR)*. Figure 4.3 diagrams the conditioning procedure.

The acquisition of a conditioned response is usually gradual; as more and more trials (CS–US pairings) are given, conditioned responses grow stronger and stronger or are more and more likely to occur. For example, in Figure 4.4, the left-hand curve shows the course of acquisition, or learning, typical in an experiment on salivary conditioning. (It is drawn without specifying the amount of saliva or the number of trials, but these values would be plotted on a graph in an actual experiment.) Figure 4.2 illustrates the course of acquisition of a conditioned eye-blink response. These are both examples of *acquisition*, or *learning*, *curves* in which the course of learning is followed over trials or time. Learning curves typically have the shape shown in the two examples; the rate of learning is

Figure 4.4

The course of conditioning, extinction, and reconditioning. Spontaneous recovery after a rest period is shown by the vertical arrow. (After McGeoch & Irion, 1952; adapted from Kimble et al., 1980.)



rapid at first but gradually decreases, as shown by the flattening of the curve. Such curves are said to be negatively accelerated. In other words, the increase in learning on later trials is less than was the increase on earlier trials. This is probably because there is a limit on the strength, or magnitude, of a conditioned response in a given experiment; after all, in the salivary conditioning experiment, the dog could only drool so much. Increases can be great on the early trials, but there is less and less to be added to the magnitude of the response as conditioning proceeds.

Instrumental Conditioning

Suppose your friend, who knows you are taking a psychology course, asks for advice on how to teach her young children to behave politely at dinner, watch less television, and do their household chores without constant prodding. What can you tell her that will be helpful? You know about classical conditioning, but that is not going to help much. For instance, how could the children be classically conditioned to say "thank you" when appropriate? What is the unconditioned stimulus for this or for other social behaviors? You need another form of learning to shape, or

Application 4.1

BEING BRAVE AND STAYING COOL WITH CLASSICAL CONDITIONING

Behavior therapy (Chapter 16, page 693) draws on learning principles in order to help people who have behavioral problems, and some of the principles behind behavior therapy come from classical conditioning. Here is an example of what is called *systematic desensitization*.

A young woman sought treatment for an intense fear, or phobia, of hypodermic syringes and needles. This fear apparently originated at the age of 2, when she received a series of shots for an intestinal disorder. From then on, she was extremely afraid of syringes and needles; and, on those few occasions when she needed routine inoculations, she had to be physically restrained. The "last straw" occurred in an emergency room where she was being treated for an injured ankle. Her struggling resulted in the doctor breaking two hypodermic needles in her ankle while trying to treat her. This convinced her that she needed treatment.

The first step in her systematic-desensitization treatment was teaching her to relax during the therapy sessions; remaining calm and relaxed is essential to this form of therapy. The standard form of systematic desensitization requires that the patient form vivid images of the feared objects or situations. Some of the images result in strong fear reactions, others produce weak responses, or no response at all. The images can thus be ranked from "strong" to "weak" in terms of the amount of fear they produce.

Since this patient was not able to generate vivid images of hypodermic syringes and needles, the therapist had to use real syringes and needles, arranging their presentation in such a sequence that the first stimuli produced no fear and later stimuli produced more and more fear. An example of a stimulus with low fear-producing potential was a brief glimpse of a small syringe held in the hand of the therapist at some distance from the patient. By varying the size of the syringes and needles, the length of time they were seen, and their distance from the patient, stronger and stronger fear stimuli were produced.

It is crucial in this therapy that the patient remain relaxed throughout the presentation of the stimuli, thus substituting relaxation for fear as the conditioned response. Having learned to relax in the presence of a weak stimulus, the next-stronger stimulus in the series was presented to the patient until she learned to relax when this one was present. This process continued until the patient could touch herself with a needle and remain relaxed. If there was a fear reaction to some of the stronger stimuli, as there often was with this young woman, the therapist dropped back to weaker stimuli before moving on again.

When the therapy was over after several sessions, the patient was no longer afraid of needles, syringes, or injections and remained calm when blood was drawn from her arm. A follow-up 10 months later showed that she was still free of fear and that no other phobia had developed to replace the old one.

You can do your own systematic desensitization. Suppose you are anxious about a tough examination that you will soon be taking. You have prepared for it, but you are still worried. One way to deal with your anxiety is to make a hierarchy of your thoughts and images about the examination, ranging from those that are weak anxiety provokers to those that are strong anxiety provokers. A weak stimulus, for instance, might be thoughts about the first day in class; then you might think about topics in the course that you liked; then think about studying for the exam; then think about what it will be like to walk into the examination room; as a next step, imagine the examination being handed out and reading the questions; and, finally, think about writing the answers to the questions. Lie down and relax while forming these images, and, should you become upset, go back to thinking about a less anxiety-provoking situation until you can remain relaxed while imagining stronger ones.

REFERENCE

Turnage, J. R., & Logan, D. C. (1974). Treatment of a hypodermic needle phobia by *in vivo* systematic desensitization. *Journal of Behavior Therapy and Experimental Psychiatry*, 5, 67–69.

mold, such behavior. To make the desired responses more likely and the undesired ones less likely, you need a set of techniques that can be applied to the ongoing behavior of the children. The techniques of instrumental conditioning will help do just that.

Instrumental conditioning is called *instrumental* because, as we will describe in detail, the key feature of this form of learning is that some action (some behavior) of the learner is instrumental in bringing about a change in the environment that makes the action more or less likely to occur again in the future. For example, if the environmental change is a reward, the instrumental behavior that brings about the reward will be more likely to occur in the future. In other words, if the behavior pays off, it is likely to be repeated.

Instrumental conditioning is sometimes also, roughly speaking, known as *operant conditioning*. This term was coined by B. F. Skinner to indicate that when a response operates on the environment, it may have consequences that can affect the likelihood that the response will occur again. We will use this term when describing the work of Skinner and others who use the methods pioneered by him.

REINFORCERS AND PUNISHERS

An environmental event that is the consequence of an instrumental response and that makes that response more likely to occur again is known as a *reinforcer* or a *reinforcement*. A *positive reinforcer* is a stimulus or event which, when it follows a response, *increases* the likelihood that the response will be made again (Figure 4.6*a*). It is important to note that the reinforcer is contingent upon the instrumental response. In other words, the response results in the occurrence of the reinforcer. This should not be a strange idea to you; responses which pay off in some way are likely to be repeated. For instance, food for a hungry animal, water for a thirsty one, praise from a parent, a prize, and many, many other stimuli or events will serve as positive reinforcers when they are contingent on some behavior.

Negative reinforcement is another tool in the kit of the instrumental conditioner. A *negative reinforcer* is a stimulus or event which, when its

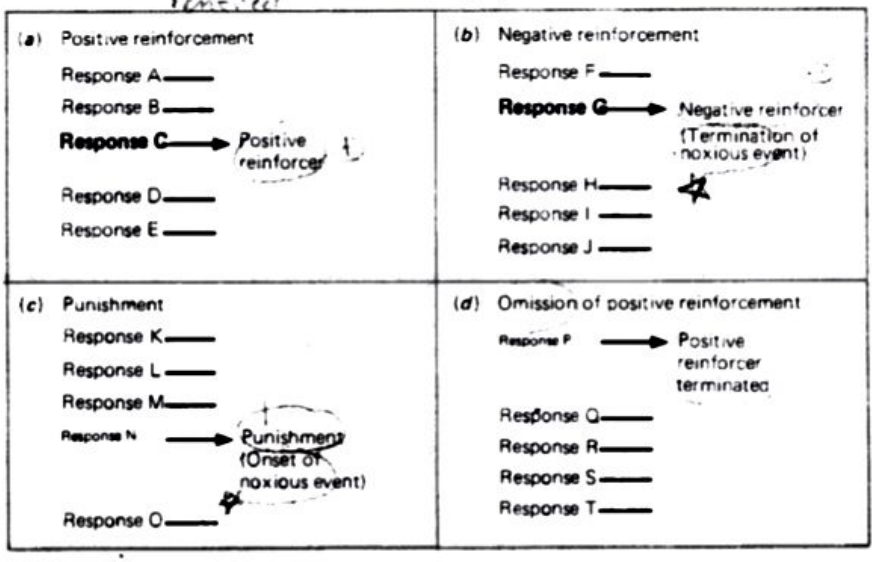


Figure 4.6

In box *a*, a response that results in positive reinforcement becomes more likely (shown by the larger type); other responses do not. In box *b*, if negative reinforcement is contingent on a response, the response becomes more likely (shown by the larger type). Boxes *c* and *d* show that responses resulting in punishment or omission of positive reinforcement decrease in likelihood (shown by smaller type).

cessation or *termination* is contingent on a response, *increases* the likelihood that the response will occur again (Figure 4.6*b*). The word *negative* refers to the fact that the response causes the termination of an event. Again, this concept should not be strange to you. Negative reinforcers are often (but not necessarily) painful, or *noxious*, events—an electric shock, for example, or a bawling out from the boss. The payoff is that the response, when made, stops the noxious event (the shock or the boss's abuse). We tend to repeat responses that pay off in this way.

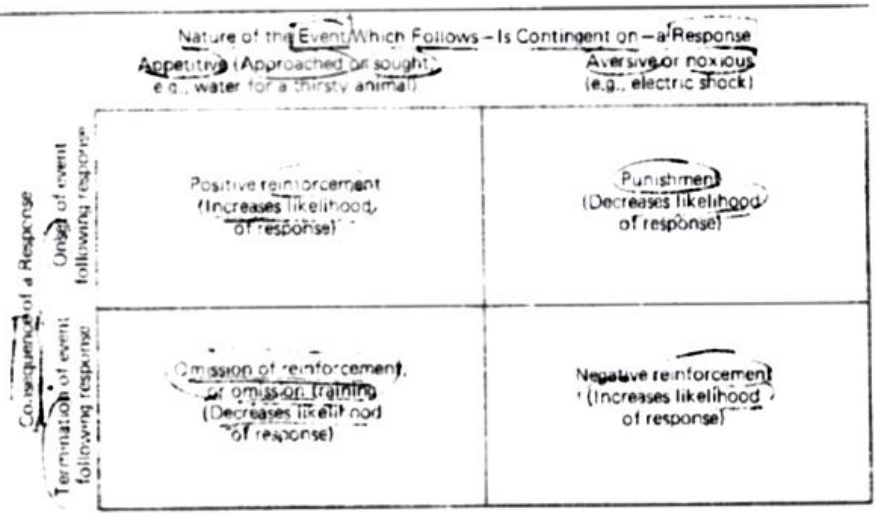
Another technique that can be used in instrumental conditioning is *punishment*. A *punisher* is a stimulus or event which, when its onset is contingent on a response, *decreases* the likelihood that the response will occur again (Figure 4.6*c*). Note that a punisher is quite different from a negative reinforcer. While both are often noxious events, it is the ending of the event that is contingent on the response in negative reinforcement, but it is the onset of the event that is contingent on the response in punishment. Responses followed by punishers tend not to be repeated, while those followed by negative reinforcers tend to become more likely. The idea of punishment is probably all too familiar. Traffic fines, demerits, spankings, and the like can act as punishers when they are contingent upon particular behaviors.

A fourth response consequence sometimes used in instrumental conditioning is known as *omission of reinforcement*. In omission of reinforcement, or *omission training*, as it is often called, positive reinforcement is withdrawn following a response (Figure 4.6*d*). The effect of the omission of the reinforcement is to *decrease* the likelihood of the response that led to the removal of positive reinforcement. An example of this is parents' turning off the television set (a positive reinforcer) and sending a child to his or her room following some behavior. In everyday speech, this is sometimes referred to as "punishment," but from the definitions given here, you can see that, technically speaking, there is a difference between omission training and punishment.

So instrumental, or operant, conditioning has quite a few ways of changing behavior. When you have mastered these ideas, you should have something useful to say to the hypothetical friend in the opening paragraph of this section. We will not go into omission of reinforcement,

Figure 4.7

Contingencies in instrumental, or operant, conditioning. (See text.)



but we will use the concepts of positive reinforcement, negative reinforcement, and punishment to organize the rest of this section on instrumental conditioning. Figure 4.7 is a summary of the contingency tools used in instrumental, or operant, conditioning. Now would be a good time to look back at the example that opened this chapter. What were the instrumental conditioning techniques used in the treatment of Billy?

Humanistic Theories: Personality as the Self

If we stop studying other people's behavior for a moment and pause to think about our own, we become aware of a set of feelings and attitudes and a certain sense of who we are. A number of theorists have focused their work upon this entity known as the *self*. Generally speaking, the term *self* has two distinct sets of meanings. One set has to do with

Figure 14.11

Our self-image develops on the basis of information about the way we are and the way others see us. (Ken Heyman.)



people's attitudes about themselves; their picture of the way they look and act; the impact they believe they have on others; and their perceived traits, abilities, foibles, and weaknesses. This collection constitutes what is known as the *self-concept*, or *self-image*, "attitudes, feelings, perceptions, and evaluations of . . . self as an object" (Hall & Lindzey, 1970, p. 516). (See Figure 14.11.) The second set of meanings relates to the *executive functions*—processes by which the individual manages, copes, thinks, remembers, perceives, and plans. These two meanings, *self* as "object" and *self* as "process," are seen in most theories involving the notion of self.

We have already considered one concept similar to the "self as process." In psychoanalytic theory, the ego comprises the personality's executive functions. In fact, for many of the "ego psychologists" who came after Freud, the ego plays *the* paramount role; Freud's own daughter, Anna Freud (1946), was a leader in this movement. Self-concepts are central to *humanistic theories*—theories which focus upon the individual's subjective perceptions of self, the world, and the self within the world. Two of the most influential humanistic theorists are Carl Rogers and Abraham Maslow.

ROGERS' SELF THEORY



Carl Rogers' theory (Rogers, 1959, 1961) grew out of his client-centered approach to psychotherapy and behavior change (see Chapter 16, page 688). Like psychoanalysis, the theory grew from efforts to help troubled people, but Rogers' theory does not involve complex personality structures or stages of development. Instead, Rogers emphasized the whole of experience, the *phenomenal field*. This is the individual's subjective frame of reference; it may or may not correspond to external reality.

The Self Out of the phenomenal field, a self or self-concept gradually develops. Rogers did not start out to make the self a central idea in his theory, but he found that clients spontaneously thought in such terms. "It seemed clear . . . that the self was an important element in the experience of the client, and that in some odd sense his goal was to become his 'real self'" (Rogers, 1959, p. 201). Thus, in addition to the present self, there is also an *ideal self*, the self the person would like to be. Trouble occurs when there are mismatches, or *incongruences*. For example, the perceived self may not match the ideal self, and this can be very disturbing. Because we need self-esteem, we may distort our perception of our experiences in self-serving ways. In doing this, we may use conditions of worth—characteristics of our behavior that we learned in childhood to associate with acceptance (versus disapproval). This brings us to one of Rogers' key notions about personality development.

Personality Development As children grow, parents and others react to their behavior, sometimes in a positive way and sometimes with disapproval. Children thus learn to regard some of their actions, thoughts, and feelings as unworthy, and they often react by distorting or denying these unworthy aspects of self.

Consider Dollard and Miller's "Mrs. A." for example (page 593). She had grown up in an atmosphere of severe strictures against sexual expression of any kind, even verbal expression. Her foster mother repeat-

edly emphasized that sex was wrong and "dirty." She did not even permit terms like *pregnant* to be used in the house. Instead, she referred to pregnant women as "big." Thus, Mrs. A. grew up regarding her own sexual thoughts and feelings as wrong, "dirty," and unworthy. She learned to cope by oscillating between distortion on the one hand and denial on the other. She distorted by sometimes adopting the view that she was a "dirty," evil person and proving her point by going on binges of flirtation and sexual seduction (the truck-driver episodes, for example). She denied by sometimes behaving in an overly straightlaced, conservative, asexual manner (for instance, in relations with her husband). Neither pattern—distortion or denial—was an accurate reflection of Mrs. A.'s true self. Consequently, both patterns led to maladaptive behavior and eventually unhappiness. Rogers believes that in mature, adjusted people, there is congruence between the total person and the self and that well-adjusted people can accept the full range of their experiences without distorting or avoiding them.

Rogers encouraged research to test his hypotheses. He was, in fact, among the first to subject the intimate experience of psychotherapy to the cool eye of evaluation when he began tape-recording sessions so that they could be coded and evaluated. He also helped to popularize the *Q-sort technique*, a method of self-description, in personality research. In this technique, the individual is given a large number of descriptions and is asked to sort them into categories from "least characteristic" to "most characteristic." A person could sort the statements, for example, to indicate "how I am now" and "how I would like to be," yielding a measure of the degree of congruence between the perceived self and the ideal self.

Research findings have supported some, but not all, of Rogers' theoretical notions. In support of his theory it has been found that people who are seeking psychotherapy indicate more of a disparity between their real and ideal selves than do people who are not seeking help. Moreover, this disparity tends to shrink in the course of successful therapy. On the other hand, some findings have suggested that acknowledging a disparity between one's real and ideal self may be a sign of maturity. Among children and adolescents, for example, it has been found that disparity

MASLOW'S SELF-ACTUALIZATION THEORY



Another phenomenological theorist who emphasized development of the self was Abraham Maslow. Maslow believed that each person has an essential nature that "presses" to emerge, like the "press" within an acorn to become an oak tree. In his view, we all have higher-level growth needs—such as the need for self-actualization and understanding of ourselves—but that these higher needs only assume a dominant role in our lives after our more primitive needs (physiological needs, safety needs, needs for love and "belongingness," and self-esteem needs) are satisfied (see Figure 14.12). (See also Chapter 7, page 298.) The growth needs, Maslow believed, help make us distinctly human. Maslow stressed that "the human being is not a white rat" and emphasized that "man has a higher and transcendent nature" (Maslow, 1971, p. 349).

To understand this transcendent nature, Maslow believed we should study transcendent people. He worried that the exclusive study of emotionally disturbed people—as in Freud's theory and others—was





Figure 14.12

(*Left:*) Maslow's hierarchy of needs. Shown at the bottom are basic biological needs; at the top are distinctly human psychological needs. Needs lower in the hierarchy must be met before "higher-level" needs can assume a dominant role. (*Right:*) In seeking examples of *self-actualized people*—people who functioned at the upper level of the hierarchy—Maslow looked to such "optimal" people as Abraham Lincoln, shown here with his son, Todd. (Photograph by Mathew Brady from The Library of Congress.)

bound to produce a distorted psychology. Instead, he studied models of *self-actualized people*—people who appeared to have fulfilled their basic potentialities. He found some of his subjects in history (Lincoln [Figure 14.12], Jefferson, Thoreau, Beethoven) and others from among his contemporaries (Eleanor Roosevelt, Einstein, a friend who was an unusually creative housewife, another who was a clinical psychologist, and others who were in business, sports, and the arts). Maslow (1967) found that this group of "optimal" people shared some distinguishing characteristics.

1. They were open to experience "vividly, selflessly, with full concentration and total absorption."

2. They were in tune with themselves, their inner beings.

3. They were spontaneous, autonomous, independent, with a fresh, unstereotyped appreciation of people and events.

4. They devoted total effort to their goals, wanting to be first-rate, or at least as good as they could be.

5. They were dedicated, fully and creatively, to some cause outside themselves.

6. They related to a few specially loved others on a deep emotional plane.

7. They resisted conformity to the culture; they could be detached and private.

Few people can be labeled *self-actualizing* in this complete sense. Yet most of us have had moments of true self-actualization, or what Maslow referred to as *peak experiences*—a burst of insight, a betrothal, the birth of a baby (Figure 14.13), a mountaintop sunrise. During these highly focused, vivid moments, there is often a disorientation in time and space, a feeling of richness and unity. The accompanying emotional reaction "has a special flavor of wonder, of awe, of reverence, of humility and surrender before the experience as before something great" (Maslow,



Figure 14.13

Each of us has had peak experiences that transcend our usual lives and approach what Maslow called *self-actualization*. (Birth photo from Hella Hammid/Photo Researchers; hang-gliding photo from Peter Menzel Stock, Boston.)

1968, p. 82). "The person at the peak is godlike not only in senses . . . but . . . in the complete, loving, uncondemning, compassionate and perhaps amused acceptance of the world and of the person" (pp. 87–88).